

MedBank

Product & positioning brief

medbank.com.ng · a study platform for medical students · prepared for the positioning, pricing and go-to-market conversation

How to use this document. Paste this into a fresh conversation to discuss positioning, pricing, go-to-market and the investor narrative. No code, no repository access — a separate workstream owns the build. Push back hard: it is cheaper to find the holes here than in front of an investor.

What MedBank is

A study platform for medical students, built around one observation: **you are examined on your own lecturer's notes, but every study app on earth is built on a generic global dataset.**

MedBank takes the actual lecture — a PDF of the slides, photographs of a handwritten note, a recording of the class, a YouTube link, or pasted text — and turns it into a complete, personalised study system: a clean note, a simplified version, flashcards, clinical questions, an active-recall engine, explainer videos, a podcast, and 3D anatomy.

Live as a progressive web app at medbank.com.ng. Works offline, installs to the home screen, syncs across devices. Built for Nigerian MBBS students first — 500-level, lecturer-specific curriculum — and designed to export.

Complete feature inventory

1 · Getting content in

- **Import a lecture four ways** — PDF, photographs of notes including handwritten, a YouTube link with captions, or pasted text
- **Record the lecture live** in-app; the audio is transcribed and becomes the study set
- **Note builder** — write or edit notes directly
- **Lecturer management** — content is organised by course *and by the lecturer who taught it*, because that is how exams work
- **Folders, search, subjects** — the student's own filing system

2 · What gets built from a single import

- **The note** — cleaned, structured, outlined
- **Simplified note** — the same content at a lower reading load, for first contact or panic revision
- **Primer** — the orientation pass before detail
- **Flashcards** — with spaced repetition
- **Clinical questions** — refined and personalised from that lecture, not generic

- **Cram sheet** — the night-before compression
- **Written tests** — long-form answers, not only multiple choice

3 · The learning engine — the real product

- **Active recall** — you produce the answer; recognition is never allowed to masquerade as knowledge
- **Smart Drill** — the core adaptive engine (frozen, stable)
- **Gap loop** — a miss is not merely marked wrong. It becomes identify → learn → practise → retest, as a tracked state machine
- **Post-session fix queue** — “here are the three things to fix” after every session
- **Retest pool** — everything missed returns on schedule until it is owned
- **Fragile-knowledge detection** — flags what was answered correctly but shakily
- **Misconception engine** — built, currently switched off
- **Target scheduling** — what to study, in what order, today
- **Session budget** — study in fifteen minutes between wards, not only in three-hour blocks
- **Mock exam** and **Solve** — full exam simulation
- **Mega Q-bank** — the large cross-topic bank
- **Progress / This week** — where the student stands
- **Study timer** with streaks

4 · Visualize — turning text into pictures

- **Highlight → Visualize.** Highlight any sentence in a note and it becomes a step-by-step animated explainer with narration, built from that sentence
- **Topic preview video.** Every topic carries a short narrated multi-scene explainer generated from the note. All narration is bought before playback so it plays without gaps
- **Ask AI / tutor** — ask about anything, in context
- **Podcast** — a two-host study episode generated from a lecture, in natural cloud voices (premium)
- **Read-aloud** throughout, in one consistent voice across the whole app

5 · 3D anatomy — the newest layer

- **The model appears inside the note being read.** Not a library to go and search — a note about the vertebral column grows a 3D chip, and the column is right there
- **Highlight a structure → see it in 3D,** opened focused on that structure
- **Guided traces with voice.** “Follow the long head” flies the camera from the supraglenoid tubercle, through the intertubercular groove, to the radial tuberosity — narrating each stop, and waiting for the sentence to finish before moving on
- **Landmarks painted on the bone** in flat atlas colours with hard edges: an origin is a coloured area, not a floating dot, so the student can see *how much* of the bone the attachment takes

- **Measured, not estimated.** Landmark positions are derived from the mesh geometry itself and verified — a point claiming to be inside the vertebral foramen is checked to be in open space, not in bone
- **Isolate, ghost, cross-section, compare, peel layers, tour parts**
- **Honest gaps.** Where the source catalogue holds no model — no spinal cord, no lymph nodes, no mammary tissue, no nerves beyond the optic nerves — the scene records it rather than faking it, and stays hidden from students until it can teach properly
- **Built for a Nigerian phone.** Meshes compressed roughly twelve-fold; a complete thirty-four-piece vertebral column is about 7 MB rather than 18
- **Status:** 14 scenes live, 34 authored, 142 meshes hosted. An automated authoring task adds scenes hourly against a 207-structure curriculum. Anatomy from BodyParts3D, © DBCLS, licensed CC-BY-SA 2.1 JP

6 · Infrastructure

Cross-device sync (Supabase) · offline-first PWA · backup and restore · paywall and entitlements via Paystack · geographic pricing tiers · usage quotas per plan · telemetry throughout.

What the student actually experiences

Monday. The lecture ends. They photograph six pages of handwritten notes, or record the class. Two minutes later they have a clean note, a simplified version, forty flashcards, clinical questions and a cram sheet — from *their* lecturer's material.

Opening it. A short narrated video at the top explains the topic before they read a word. Reading, they hit a sentence that will not resolve — they highlight it, and it becomes an animated explainer with narration. Further down, a note on the vertebral column carries a 3D chip: the column appears, rotatable, every disc in place, and they follow the spinal levels while it talks.

Testing themselves. Active recall makes them *produce* answers. Every miss is caught, named and queued. At the end: "here are the three things to fix." Those three return tomorrow, and next week, until they stop being missed.

Between wards. Fifteen minutes. The app knows what is due and gives them exactly that much.

The night before. The cram sheet. Every gap they closed. And — the thing nothing else gives them — **they know what they know.**

The competitive picture

	What they have	What they cannot do
Anki	Enormous shared decks; best-in-class spaced repetition	Nothing is <i>yours</i> until you build it by hand. No content generation, no explainers, no 3D
Amboss / UWorld / Osmosis	Huge validated question banks, excellent editorial	Built on a global curriculum. Has never seen your lecturer's slides. \$20–40 per month
Complete Anatomy / Kenhub	Far better 3D than I will ever build	A separate app you leave your notes to visit. It does not know what you are examined on next week

	What they have	What they cannot do
Quizlet / StudySmarter	Easy, cheap, general	Not medical. No clinical reasoning, no depth
Local Nigerian apps	Past questions and PDFs	Static content. No generation, no adaptivity, no personalisation

The gap I sit in. Amboss and Anki own the *dataset*. I own the *relevance*. A student uses Amboss to learn medicine and MedBank to pass the exam they are actually sitting — and because the questions are clinical and refined, they get the medicine too.

“Anki gives you a deck. Amboss gives you a question bank. Neither has ever seen your lecturer’s slides.”

Combinations believed to be unique — each should be challenged:

- 3D anatomy that surfaces inside your own imported lecture note
- Highlight a sentence → instant animated explainer
- Auto-generated narrated preview video for every topic
- All of the above in one app, aimed only at medical students
- Local-first pricing with a global subsidy

Business position so far

Item	Position
AI core	DeepSeek v4 Flash, chosen for cost. Roughly NGN 50–100 in model cost per import
Assumed usage	20 imports a month works out at roughly NGN 2,000 per student; NGN 3,500 assumed to leave headroom for power users
Pricing intent	Nigeria NGN 7,000 basic / NGN 12,000 premium. International \$7–15 — advised to raise to \$19 / \$29 so it genuinely subsidises Nigeria and does not signal low quality
Competitor pricing	\$20–40 per month
Go-to-market as first planned	TikTok and Instagram ads to international students — advised this is the wrong first move . Get pilot evidence in one Nigerian school, distribute through class WhatsApp groups, then organic content showing the 3D and Visualize, then paid
First contact	A pharmacist friend is interested and knows a well-followed finance figure on Twitter he would introduce once the product is finished

What is honestly not done

No pilot evidence yet. No retention, activation or outcome numbers. This is the single biggest gap, and everything else in this document is worth more once it is closed.

- **Fully-loaded cost per user is unknown.** Model cost is known; text-to-speech, transcription, storage, egress and hosting are not yet measured. Text-to-speech is the likely sleeper cost
- **Import volume is uncapped** — a student cramming for finals could break the unit economics
- 3D: 14 of 34 authored scenes are live; several anatomical families have no source models at all
- Misconception engine built but disabled

What I want from this conversation

- Tear the positioning apart and rebuild it stronger
- Pressure-test the pricing and the geographic subsidy — including how to stop VPN arbitrage
- Sequence the go-to-market properly
- Say which of the “nobody else does this” claims will not survive scrutiny
- Write the investor narrative, and the exact numbers I must hold before that conversation
- Say what to instrument **now**, since it cannot be retrofitted later

Be blunt. If something here is weak, say so plainly.